

Policy on Nutritional Therapy

«Hospital Name»

Document control

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Prepared by	Dietetics In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Every admitted patient is screened for nutritional risk, at-risk patients get a dietician-led assessment, and the resulting diet is planned collaboratively and delivered exactly as ordered.

1. Purpose

This policy explains how «Hospital Name» screens admitted patients for nutritional risk, assesses at-risk patients, plans and delivers therapeutic and normal diets collaboratively, and educates and monitors families who bring in food.

This policy does not cover general patient assessment in detail — that is covered in other hospital policies.

2. Scope

This policy applies to all doctors, nurses, and dieticians involved in nutritional screening, assessment, and diet planning at «Hospital Name».

3. Policy standards

«Hospital Name» screens every admitted patient for nutritional risk, and a dietician carries out a full nutritional assessment for anyone found at risk. Therapeutic diets are planned collaboratively between the dietician, the treating doctor, and the patient or their relative, accounting for allergies, food habits, and preferences. Every patient has a written diet order kept in a consistent place in the medical record, and receives food accordingly. Where

families bring in food, they're educated about the patient's diet limitations, and their compliance is monitored.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not admit a patient without screening them for nutritional risk, and do not let anyone other than a doctor or nurse carry out that screening.
- Do not skip a dietician-led nutritional assessment for any patient found at risk during screening — the assessment must at minimum result in a plan for the diet's planning, preparation, and distribution.
- Do not plan a therapeutic diet without involving the dietician, the treating doctor, and the patient or their relative together, and without accounting for the patient's food allergies, food habits, and likes and dislikes.
- Do not give a patient food without a written diet order kept in a consistent, uniform location in the medical record.
- Do not let a family bring in food for a patient without educating them on the patient's diet limitations and monitoring their compliance with the prescribed diet.

5. What we do

5.1 Screen every admitted patient

Every patient admitted to «Hospital Name» is screened for nutritional risk by a doctor or nurse. This could include a validated tool — for example NRS-2002, the Mini Nutritional Assessment Short Form, MUST, or the GLIM criteria — alongside clinical assessment.

5.2 Assess patients found at risk

A dietician carries out a nutritional assessment for every patient found at risk during screening. It's preferable to use a validated tool for this. At minimum, the assessment covers the planning, preparation, and distribution of the patient's diet.

5.3 Plan the therapeutic diet collaboratively

The nutritional assessment results in a therapeutic diet. The dietician plans this in consultation with the treating doctor and the patient or their relative, taking into account the patient's food allergies, food habits — vegetarian or non-vegetarian — and likes and dislikes. The patient then receives food matching the diet order.

5.4 Deliver food according to the written order

Every patient receives food according to a written diet order — either a normal diet, for patients found at no nutritional risk, or a therapeutic diet. The order is kept in a consistent, uniform location in the medical record. For a therapeutic diet, the dietician prepares a diet

sheet, and the patient receives food matching it — for example a diabetic diet, a high-protein diet, or total parenteral nutrition.

5.5 Educate and monitor families who bring food

Where a patient's family provides food, the dietician or a nurse educates them on the patient's diet limitations — ideally at the planning stage. The dietician then monitors the family's food against the prescribed diet, to make sure it stays within those limitations.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Dietetics In-Charge	Owens day-to-day implementation; maintains written guidance; conducts or oversees dietician-led nutritional assessments for all at-risk patients; leads collaborative diet planning; ensures diet sheets are prepared for therapeutic diets; monitors compliance of families who bring food.
Treating doctors	Participate in collaborative therapeutic diet planning; write or countersign diet orders in the uniform location in the medical record.
Nursing staff	Carry out nutritional screening at admission (screeners must be doctor or nurse); educate families on diet limitations when families provide food; document screening in the medical record.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Nutritional screening	Every admitted patient screened for nutritional risk; screening done by a doctor or nurse only; documented in the medical record
Dietician-led assessment	Dietician carries out nutritional assessment for every patient found at risk; assessment covers at minimum planning, preparation, and distribution of the diet; validated tool used where applicable
Collaborative diet planning	Therapeutic diet planned by dietician in consultation with treating doctor and patient or their relative — all three parties

	involved; three patient factors accounted for: food allergies, food habits, likes and dislikes
Written diet order — normal or therapeutic	Written diet order in place for every patient; kept in a consistent, uniform location in the medical record; food delivered matching the order
Therapeutic diet sheet	Dietician prepares a diet sheet for every therapeutic diet; patient receives food matching the sheet
Family-provided food — education	Family educated on patient's diet limitations by dietician or nurse when family brings food
Family-provided food — monitoring	Dietician monitors family's food against the prescribed diet when family brings food — separate from education; both obligations must be evidenced

8. Training and staff acknowledgement

All doctors, nurses, and dieticians shall be familiar with the requirements of this policy, including: the screener role (doctor or nurse only — not delegated to non-clinical staff); the assessor role (dietician only — distinct from screener); the three minimum assessment output elements (planning, preparation, and distribution of the diet); the three-party collaborative diet planning requirement (dietician + treating doctor + patient or relative); the three mandatory patient factors in diet planning (food allergies, food habits, and likes and dislikes); the uniform location in the medical record for diet orders; and the two distinct obligations when families provide food (education by dietician or nurse AND monitoring by dietician — both mandatory).

I have read the Policy on Nutritional Therapy of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all treating doctors, nurses, dieticians, and quality staff at «Hospital Name».

10. Abbreviations

Abbreviation	Meaning
COP	Care of Patients (NABH Hospitals chapter)

GLIM	Global Leadership Initiative on Malnutrition
MUST	Malnutrition Universal Screening Tool
NABH	National Accreditation Board for Hospitals and Healthcare Providers
NRS	Nutritional Risk Screening
TPN	Total Parenteral Nutrition

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.19.a	Commitment*	Section 5.1 addresses nutritional screening of every admitted patient, done by a doctor or nurse only — not by a dietician or non-clinical staff. Validated tools (NRS-2002, MNA-SF, MUST, GLIM criteria) may be used alongside clinical assessment. Screening is documented in the medical record.
COP.19.b	Commitment	Section 5.2 addresses dietician-led nutritional assessment for every patient found at risk during screening — the dietician role is mandatory and distinct from the screener role. The assessment must cover at minimum three output elements: planning, preparation, and distribution of the patient's diet. Use of a validated tool is preferable.
COP.19.c	Commitment	Section 5.3 addresses collaborative therapeutic diet planning requiring three parties: the dietician, the treating doctor, and the patient or their relative — all three must be involved. Three patient factors are mandatory inputs: food

		allergies, food habits (vegetarian or non-vegetarian), and likes and dislikes. The patient receives food matching the diet order.
COP.19.d	Commitment	Section 5.4 addresses written diet orders for every patient — normal diet for patients at no nutritional risk, or therapeutic diet for at-risk patients. The order is kept in a consistent, uniform location in the medical record. For therapeutic diets, the dietician prepares a diet sheet and the patient receives food matching it.
COP.19.e	Commitment	Section 5.5 addresses two distinct mandatory obligations when families provide food: (1) education of the family on the patient's diet limitations by the dietician or a nurse; and (2) monitoring of the family's food against the prescribed diet by the dietician. Both obligations are mandatory and must be evidenced separately.

12. Required Records/Evidence Checklist

Nutritional screening (COP.19.a)

- Nutritional screening records for every admitted patient — documented in the medical record by the doctor or nurse who carried out the screening.
- Validated screening tool used where applicable — tool identified and documented.

Dietician-led assessment (COP.19.b)

- Nutritional assessment records for every patient found at risk — conducted by a dietician (not the screener).
- Assessment covers at minimum: planning, preparation, and distribution of the patient's diet — all three elements documented.

Collaborative diet planning (COP.19.c)

- Records showing collaborative planning between the dietician, the treating doctor, and the patient or their relative — all three parties involved.
- Records confirming all three patient factors accounted for: food allergies, food habits, and likes and dislikes.

Written diet order (COP.19.d)

- Written diet order for every patient — kept in a consistent, uniform location in the medical record.
- For therapeutic diets: diet sheet prepared by the dietician; food delivered matching the diet sheet.

Family-provided food — education (COP.19.e)

- Records of education delivered to the family on the patient's diet limitations when the family brings food — conducted by dietician or nurse.

Family-provided food — monitoring (COP.19.e)

- Records of dietician monitoring the family's food against the prescribed diet — separate from the education records.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.19.
2. Guidebook interpretation supplied for COP.19.a through COP.19.e.

Disclaimer

This policy reorganises the supplied COP.19 objective elements and approved plain-language content into policy format. No formal stop-work section is included — COP.19 is not in the accreditation stop-work proposals. The screener role (doctor or nurse only) and the assessor role (dietician only) are stated as distinct mandatory roles in the Non-negotiable rules, Section 5.1 and 5.2, the governance table, the quality monitoring table, the traceability table, the evidence checklist, and the training section — neither role may be performed by the other. The three minimum assessment output elements (planning, preparation, distribution) are preserved exactly. The three-party diet planning collaboration (dietician, treating doctor, patient or relative) and the three mandatory patient factors (food allergies, food habits, likes and dislikes) are all preserved. The uniform medical record location requirement for diet orders is stated in the Non-negotiable rules and Section 5.4. The two distinct obligations for family-provided food (education AND monitoring) are preserved separately in Section 5.5, the quality monitoring table, the traceability table, and the evidence checklist.